

THE THERAPIST–CLIENT RELATIONSHIP AND THE TRANSFORMATIVE POWER OF SELF

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The primary healing relationship in internal family systems (IFS) therapy is between the client's Self and her young, injured parts. Because I have emphasized this aspect of IFS for many years while giving far less attention to the relationship surrounding it between therapist and client, I have contributed to the perception that IFS is not a relational therapy. This perception, however, is mistaken. IFS is designed to develop the relational field within and between each participant in a parallel process. In this chapter I spotlight the ways in which IFS is a relational treatment on multiple levels and I explain why the therapist–client relationship is as important in IFS as it is in any other therapy.

NURTURANCE IN THERAPY

Due to the influence of humanist therapists such as Carl Rogers ([1951](#)) and, more recently, of attachment theory, therapists have learned to emphasize nurturance in relationships with clients. Regardless of theoretical bent, therapists in training are now taught to attune to the emotions of their clients and respond genuinely. The importance of transference is also broadly accepted. Most clinicians are aware that the therapeutic relationship can evoke the client's extreme beliefs and feelings, and that these can be transformed through a safe, correctively healthy experience with the therapist.

SHIFT TO SELF/PART AS THE PRIMARY HEALING RELATIONSHIP

Nevertheless, the way IFS prioritizes the relationship between the client's Self and parts distinguishes it from most contemporary psychotherapies. Although the therapist–client relationship is key in IFS, much of the healing happens when the Self rather than the therapist becomes the primary, loving attachment figure for a client's injured young parts. This is a conceptual shift on the roles of therapist and client, which has partly obscured the importance of the therapeutic relationship. But I also contributed to the problem initially by minimizing the importance of the therapist–client relationship in response to my own experience.

While I was in college I worked summers as an aide on a psychoanalytically oriented psychiatric unit in Chicago, and I was disturbed by how much the patients there came to depend on their therapists. I got close to several adolescents on the ward who repeated some bizarre interpretations offered by their analysts and told me how, if they did not initiate speaking, whole sessions could go on in silence. I listened to them wonder obsessively whether their analysts cared about them and I watched them seek their analyst's approval before making even minor decisions. As a result, I decided that when I grew up professionally I would find a way to avoid making my clients dependent.

When I realized in the early 1980s that everyone has a Self, I felt tremendous relief. I knew it was possible to do deep healing work without fostering the kind of intensely dependent relationships that I viewed as disempowering and iatrogenic. I thought people would walk away from therapy feeling they had healed themselves. The focus would be the client's relationship with his or her parts rather than with me. When clients had transference feelings toward me, casting me in the role of someone from their past, we would find the part who saw me that way and work with it rather than working through the issue between us. In turn, when parts of me came up, I would try to get them to step back and not interfere so I could hold Self-leadership for my client rather than bringing my countertransference into our relationship.

Although some of my desire to rely on the Self of the client rather than on the therapist–client relationship was based on solid observations and principles, some of it was based on extreme beliefs that came from my own parts. Like many men, I had a part who hated being clung to and recoiled from neediness with its implicit demand that I fix things. When I disappointed clients and sensed their anger, I had parts who felt imposed on and shamed, and my reactions were intense. As a result, as I developed the model I downplayed the importance of the relationship and did not fully consider the impact of therapists who were not able to maintain Self-leadership. Since those early days, I have learned from my clients, and from the experience of being a client, that the therapy relationship is crucial. This chapter is an attempt to remedy my early mistake.

THE IFS VERSION OF SAFE CONTAINMENT

Many clients enter therapy with their protectors on high alert. They know that therapy involves being vulnerable and trusting the therapist, and they have had terrible experiences with people (including former therapists) they trusted. If the therapist can hold Self-leadership from the first session, the client's protectors will immediately sense the safety. As Ann entered Larry's office for her first session she heard a familiar inner voice critiquing his taste in furniture and clothes. This part of her also said his handshake was weak and he looked like an old hippie. In her head she heard, "He won't be able to handle you—you should leave right now."

At the same time, Ann could not help noticing that he had friendly eyes and an easy, calm demeanor that felt different from the previous two therapists she had fired. As she described her dreadful history to him—the shameful suicide attempt and nightmarish hospitalization, the past bouts of depression and current emptiness—she scanned his face for signs of disdain or fear but found none. Instead, he asked questions about the "parts of her" who had made her depressed, empty and suicidal—what types of things had they said to her or made her feel—and how did they seem now? His voice betrayed no anxiety or judgment. Instead, Larry seemed genuinely curious about these parts.

Her protector decided to test him. "What do you mean parts?" she said with no little contempt. "Are you implying that I'm DID?" She knew all the psychiatric terms. He explained calmly that he was using a therapy model that believed we all have parts and there are no bad ones—just ones who are hurt or protective. He also said the model was effective in helping all parts to unload any pain, fear and shame they carry and, once they have unloaded it, to return to their naturally valuable states.

Her contempt did not seem to intimidate or provoke him and although this confidence raised hope in some of her parts, it scared others. The hope came from a sense that he understood. Ann knew that her tormenting inner voices and urges had a life of their own, which had always made her feel crazy. If he could deliver on the promise that these parts were not what they seemed—and they could change—it would mean liberation from constant torment.

On the other hand, if he could not deliver she would be devastatingly disappointed and would likely become suicidal again. After all, he had not yet met her incredible neediness and he did not know about the shameful things that had happened to her or why she deserved them. Maybe she was the defective exception, or maybe he was not as good at therapy as he was implying. While this debate raged inside her, Larry said something startling: "It looks like you have a part who's skeptical about what I'm saying and I want you to know, if that's true, I honor that part and won't go any further without its permission. I know I have to earn its trust and I'm happy to discuss all its fears."

In this brief exchange, Larry not only communicated curiosity, calm and confidence (three of the 8 Cs of Self-leadership), but also unflappable equanimity despite the severity of her history, symptoms and provocative challenges. He was a hope merchant, presenting a vision of new possibility to her pessimistic system. In addition, he was solicitous of her protector and respectful of its need to make sure that his approach would be safe.

Safety in IFS derives from a combination of potent ingredients. First, the therapist's unaffected warm, steady, accepting *presence*, fortified by the IFS *perspective* that allowed Larry to exude hope and

confidence because he trusted that her severe symptoms were a product of extreme parts carrying pain that could be unloaded, rather than being evidence of scary pathology. Next, he showed *patience* and respect for what might be considered her resistance, which he viewed as the understandable need of her protectors to ensure that he was safe. These qualities instilled seeds of hope in Ann, while reducing her fear and frustration with her own parts. Because the therapist and client are in a parallel process, clients such as Ann increasingly demonstrate patience, perspective and presence with their own parts—they come to relate to their parts just as the therapist’s Self relates to them.

THE 5 PS

In other words, Larry’s Self-energy elicited Ann’s, as one tuning fork wakens another. Ann’s protectors sensed this safety and relaxed, allowing her Self to surface. As the therapy continued, she increasingly manifested more of the core qualities of Self-leadership, which I call the “8 Cs.” In addition to curiosity, confidence and compassion (mentioned above), the others are *courage*, *clarity*, *creativity*, *calm* and *connectedness*. Over time, Ann displayed more of these signs of Self-leadership both in her relationship with Larry and with her parts.

The ability of the therapist to manifest those 8 Cs—to maintain Self-leadership—is also central in creating the safety and acceptance that characterizes the therapeutic relationship in IFS. In addition, we highlight five other qualities of the Self-led therapist, called the “5 Ps,” which are the foundation of the therapeutic relationship in IFS. I just introduced the first three: patience, presence and perspective. In later sessions Larry will demonstrate the other two: *persistence* and *playfulness*.

Ann came to the fifth session determined to talk rather than turning her attention inside. In the previous two sessions, Larry had persuaded her to begin getting to know her parts and they had started with the skeptic. As Larry talked to this primary guardian directly, Ann was surprised at how quickly it was disarmed. Larry convinced the skeptic that Ann could witness and heal her sad part, whom the skeptic protected, without being overwhelmed, which would free the skeptic to take another role in her internal system. Immediately following this discussion, Ann saw herself as a little girl being sexually abused by the son of a neighbor—a memory she had vowed many years earlier never to revisit. While Larry was able to help Ann enter the scene and take the girl to a safe place, Ann had not checked on the girl during the ensuing week as she had promised. Indeed, she remembered nothing about the previous session until she arrived for the current one. She had no idea how the girl was doing and did not want to feel guilty about having abandoned her.

She told Larry that she felt much better since their last piece of work. She was very grateful for his help and thought she could take it from here. Larry asked permission to speak directly to the part talking, and then asked the part to be honest about why it wanted to stop. It replied, “I’ve had enough mucking around in the past! This is dangerous and scary. The suicide thoughts came up again for the first time in years.”

Demonstrating perspective, Larry said, “I’m not surprised. Many clients have some kind of backlash after their first encounter with an exile.” He did not seem flustered by the return of her suicidality, and

he demonstrated presence by saying, “We just need to find the parts who made you forget about the girl.” This felt strange to Ann, who was used to being attacked and shamed. Her protectors kept waiting for the other shoe to drop—for Larry to suddenly turn on her.

THE IFS VERSION OF THE CORRECTIVE EXPERIENCE

Many psychotherapies know that deep healing takes place when a client who expects to be humiliated or rejected experiences the therapist’s acceptance and love instead. The IFS therapist’s tenacious caring helps clients take the slogan *all parts are welcome* seriously and trust that no parts are bad. In this way IFS involves more of a corrective experience than I have discussed in the past. After disclosing or manifesting their most shameful parts, clients are profoundly relieved to be met with compassion and the words “It’s just a part of you—not who you are—and even that part isn’t what it seems to be.” This is the parallel process. When the therapist responds to the client’s extreme parts from Self, the client does also. Compassion and acceptance are contagious and can seep through the cracks into all levels of the internal system.

Even better, clients learn Self-leadership is not an exercise in becoming one’s own cheerleader through affirmations (e.g., “I am a good person”), which inner critics can deconstruct seconds later. Instead, during inner work clients have an in-the-bones experience that who they really are is pure goodness and strength (the Self). After these experiences, clients who had no inkling of such internal capacities are often shocked when I point out how strong and caring they became once their parts opened space for their Self to emerge.

THERAPIST'S TRUST IN THE CLIENT'S SELF

In IFS, the client is empowered by the therapist’s complete trust that she will know what to do once she has accessed her Self. However, until the moment when the critical mass of Self manifests (which can be seen in the client’s shift in tone of voice, facial expression and word content), I am often reflecting back empathically what the client reports her parts saying, and offering suggestions for what the client might say or do with a part. This stage is quite similar to reflective listening, which most therapists learn in graduate school. Reflective listening helps the target part feel accepted by me before more of the client’s Self emerges, and it also helps her protectors trust that we are on the right path so they can relax their grip. Once a client manifests a critical mass of Self, IFS therapists learn to get out of the way, just to keep an eye out for parts of the client who might interfere, making occasional empathic comments, or sometimes asking questions like, “What do you say to the part about that?” or “Where do you want to go now?” or “What feels like the right next step?” As well as the ubiquitous Self-detecting question “How do you feel toward the part now?”

As the therapist continues to be a curious, empathic partner in exploration with the client rather than an expert who is figuring it out for the client and giving her the missing direction, the client's Self is allowed to demonstrate its abilities and increasingly her parts come to trust her own leadership rather than looking to the therapist. The restoration of trust in Self-leadership is a major goal of IFS, a goal that can be impeded by the therapist's need to be useful or in control.

Suggesting that they get to know the suicidal part before returning to the girl, Larry said, "Until we befriend that part, it will be hard for your system to relax and let you be with your exiles because they will constantly be afraid of suicide." Although Ann could understand this logic, the thought of approaching her most lethal part seemed crazy. She had worked hard to keep it locked away and he wanted her to throw the gates wide open! Larry reassured her that the process would not give the suicidal one more power. Instead the goal was to get to know the part and help it realize that there was a different way to relieve her misery. To calm all fears about approaching the suicide part, Larry had to help Ann reassure six different parts. And although several initially separated, they returned when she got closer to the suicide part. Ann expected Larry to become frustrated with the seemingly endless stream of obstacles presented by her system. A part of her was listening carefully, prepared to shut everything down at any hint of impatience.

PATIENT PERSISTENCE

In our trainings, IFS therapists learn to be patiently *persistent*. Until there is a clear message from the client's protectors to stop, they learn to consistently offer the opportunity to keep going, knowing that, in many cases, the client is being invited to do the scariest thing she has ever done. When a therapist leads from Self, she can be doggedly persistent without coming across as pushy or insistent because she is not emotionally attached to achieving the goal. Instead she is calmly holding up a hopeful possibility and inviting the client to try it out. She also conveys such respect for the client's protectors and their right to refuse entrance that they do not feel threatened by her persistence. Protectors may feel frightened about opening the door to exiles but not feel additional pressure from the therapist's need for them to do it. Instead, protectors appreciate the therapist's concern along with her recognition that their fears must be addressed before entrance is permitted.

IFS therapists persist in many other ways as well. For example, the session after an exile is unburdened, the therapist remembers to follow up and see if the part is still doing well, even if the client wants to start with something else. In addition, in the face of provocative testing from the client's protectors, the therapist persists in responding with Self-energy or, if triggered, works to find her own upset part, speak for it, and return to Self-leadership. We call this tenacious caring.

PLAYFULNESS

The final P, *playfulness*, is an attitude; a lightness of being that accompanies Self-leadership. To paraphrase Oscar Wilde (1893), life is too important to be taken seriously. The Self-led therapist feels compassion for the client's suffering and therefore takes it seriously but, at another level, knows that her suffering will be lifted by the work and is often amused by the dance of the client's protectors as they try to distract, deter, dominate and divert both client and therapist. Despite how wise, dangerous or powerful they seem, most protectors are young. Like children who do not think they will be heard if they are direct, protectors believe they have to manipulate. From the position of Self, those manipulations often seem more funny or cute than threatening.

The therapist's amusement with protectors can help the client take them less seriously, too. I often remind clients that these parts are really scared, desperate children who need reassurance and firmness. Once separated from the fear of protectors, clients can often relate to them playfully. Of course, the therapist is not always playful. If the therapist's amusement is interpreted as mocking or trivializing, clients can lose trust. Therefore a therapist finds a balance between serious intensity and a bit of comic relief that fosters connection and perspective. And with some clients, the risk that they will misinterpret humor is so high that one only chuckles inside, privately.

CONNECTION TO AND TRUST IN CLIENT'S SELF

Playfulness requires a level of safety and connectedness in a relationship such that each person feels free to have fun with the other. There is something beautiful about being present when clients' Selves emerge and I witness their courage, grace and vulnerability. When my Self is fully embodied during those sessions, I feel a deep, pure loving connection to my clients and many clients report feeling the same. We do not have to talk about it to feel it. The Self-to-Self connection is wordless, joyful and often brings tears to my eyes.

I am convinced that this transcendent sense of connection is a key to the healing process. The more my client feels it with me, the more she can feel it with her parts, which is what they long for (what we all long for, for that matter). The more the client feels it, the more emboldened she is to enter terrifying inner chasms and abysses, knowing she's not alone. And the more she trusts that we are energetically connected even when not together, the more she can take risks in the outside world.

There is one more aspect of IFS that therapists trained in other therapeutic approaches say they have trouble trusting. Therapists who do not believe that the client feels their presence when they are silent will tend to keep talking while the client is working inside, asking how things are going or giving frequent direction. A high level of therapist activity such as this is generally unnecessary and can have a couple of negative consequences. First, while exploring inside, many clients have trouble remaining focused when they are interrupted. I often suggest to therapists in our trainings that they use the acronym

WAIT, which stands for “Why am I talking?” Second, the more the therapist checks to see how clients are doing or tells them what to do, the less their parts trust that their Self is capable of leading. In other words, many clients’ parts have little or no trust (or even awareness of) their Self and as long as they can look to the therapist for guidance, they will never learn how wise, strong and caring their own Self is.

In some of my sessions there are either long periods of silence while clients work inside or times when only the client is talking, describing what she is doing in the inner world. As long as I trust that the client’s Self is actively present and she is not being distracted or sent on a wild goose chase with protectors, I remain a quiet witness, extending loving energy silently. For many clients, however, I am not entirely quiet and make frequent brief empathic comments like: “yes,” “that’s right,” “I get it too” or “of course” to let them know I am with them, and I understand what happened and how bad it was. Although I can be quiet for long intervals, as long as I am fully present and openhearted clients often report sensing my presence powerfully while they explore inside. I also find that the silent presence of a group of Self-led observers, such as a training context or group therapy, bolsters clients’ access to Self-energy. However, if a client has parts who are bothered by long silences and the client wants me to be verbally present on the journey, I accommodate.

Ann focused on her suicide part and said warily, “It’s a dark, menacing presence.” The parts who feared this impulse toward suicide kept jumping in like a tag team, making Ann either terrified of the part, furious with it, thinking about why it was there or confused and blanking out. With each of these appearances, Larry chuckled and said things like, “Oh, I see! The part who takes your brain is back. See if it’s willing to step back and trust us just to get to know this suicidal one.” Larry finally suggested that she put the suicide part in a contained room in her mind. Once it was contained, her other parts relaxed and, when Larry asked how she felt toward the part, Ann replied, “It looks so sad. I feel sorry for it.”

Larry said, “Now it’s safe to begin to get to know the suicide part. Ask through the walls of the room what it wants you to know.”

Ann reported that the part, who now looked like a keening woman, was tired of being in pain and wanted out. In response Ann wanted to enter the room and comfort the woman, so Larry told her to go ahead. From that point on, Larry simply listened to Ann describe her loving interactions with the woman, who was grateful to be heard finally rather than pushed away, and was welcoming Ann’s attention and hugs.

With about 10 minutes left in the session, Larry let Ann know that they would have to stop soon and had her ask the woman what she would need from Ann until their next session. The woman said she just needed to know that Ann would return. After closing with the woman, Larry asked Ann if she had noticed how much she had taken over during the last half of the session and how she knew just what the woman needed. Ann had noticed and was shocked by how well the session went. But she also had parts telling her that it was not real—that she had faked it all to comply with what Larry wanted. “I’m not sure what happened,” she said.

THERAPIST AS LOVING COMPANION

I have been emphasizing how the therapist in IFS moves as quickly as possible to be maximally present but minimally directive. I also want to discuss the other, more active side of the therapist–client relationship. With some clients we go inside almost every session but with others there is a balance between sessions where we talk about the client’s life or problems and sessions where the client is inside most of the time. Or the focus may shift fluidly from the outside (i.e., discussing a dilemma in a relationship with a boyfriend and giving some advice) to having the client find the parts who come up as she thinks about the dilemma, shifting the focus inside to work with them. The session may end with the client thinking about the dilemma again seeing how her perception or plan-of-action changed. In a few rare cases where inner work is too threatening, we go inside infrequently and talk, though this balance between talking and inner work may shift at different points in the therapy.

Many therapists believe that they are only doing IFS when the client is focused internally, but I believe that anytime you relate to the client from your Self, and you keep in mind her system of parts, with or without using parts language, you are doing IFS. As with other therapies, the healing connection between you and your client is also fostered by a high level of interest in the details of her life and how you remember those details. It is fostered by your explicit support for the risks she starts taking, by your acceptance of her perceived failures and the parts she’s ashamed of. It is also fostered by collaborative strategizing to handle difficult relationships or predicaments, by your excitement about her successes and, when appropriate, by your openness about your own parts, including apologizing when your parts get in the way. In other words, the IFS therapist forms a collaborative relationship with the client so that the client senses she is truly cared for and is not alone in her inner or outer journeys. The more clients sense that you care about their lives; the more their protectors relax when you do go inside. This way of being with clients is, in my view, key to the success of any therapy.

Another important therapist role is helping clients achieve clarity. When a client views the world through a distorted lens because she is blended with an extreme part, she often cannot access a realistic perspective on events in her life. For example, when she gets into conflict with someone, she automatically assumes she was wrong, or the opposite—that the other person was wrong. If the therapist can maintain Self-leadership and view the client’s predicaments clearly and compassionately, then, by simply sharing a less distorted perspective, he can help the client separate from her parts and see the events from her Self. Once the client has access to her Self, she can reassure her parts instead of blending with them. Bateman and Fonagy (2004) called this ability to step out of one’s emotions and gain perspective *mentalizing*; Main (as cited in Fonagy, 1996) called it *meta-cognition*. It is now widely accepted as an important quality of secure attachment. Siegel (2011) described the same ability in the practice of mindfulness, a skill taught in many modalities of psychotherapy.

There is, however, a risk in this kind of perspective sharing. When a client’s parts are highly attached to their burdened viewpoint, they can become polarized with the therapist. The goal for the IFS therapist is to share his own perspective from Self, without being attached to the client’s agreement and with

respect for the parts who need to disagree. And then the therapist helps the client get to know and heal the parts who disagree. The challenge in this process is for the therapist to remain humble with her perspective, knowing that sometimes the client's seemingly extreme view contains some reality.

As I write this, however, parts of me worry that beginning IFS therapists will be more comfortable talking with clients rather than leading them inside and will use these paragraphs to justify avoidance of parts work. So I do want to stress that the balance between an inner and outer focus should be determined by what seems best for the client rather than by collusions between scared protectors in both client and therapist. It is tempting for therapists who are insecure in their inner skills to be relieved when the client does not want to go inside. They can both retreat to the safety of a familiar external focus.

THE EMERGENCE OF EXTREME PARTS

As clients feel a growing connection with their therapists, parts who do not ordinarily show up with other people will be drawn to make an appearance by the safety and acceptance of the relationship. Clients do not usually expose those parts to the world because they offend, scare or enrage other people. Some of these hidden parts are clearly related to what most of the field calls transference. They are frozen in hurtful times from the client's past and are concerned by the growing intimacy of the therapeutic relationship or by certain similarities between interactions with the therapist and a hurtful caretaker.

Other appearances by extreme parts are not necessarily related to transference. These parts come out just because it feels safe and they yearn for external contact or expression. The Self-led therapist is implicitly issuing an invitation to the client: "All parts are welcome!" And from the darkest corners of their psyches come the hidden aspects of clients in all their crazy glory. And that's a good thing. The inescapable reality of therapy is that if we do our job well some clients will do all kinds of provocative things and will repeatedly test us. They will resist, get angry and critical; become hugely dependent; talk incessantly; behave dangerously between sessions; show intense vulnerability; idealize us; attack themselves and display astounding narcissism and self-centeredness.

Therapists must learn to be mindful of their own parts as they work with clients, recognizing that transference and countertransference form a continuing behind-the-scenes interaction as therapists and clients inevitably evoke each other's protectors and exiles. When therapists are not overwhelmed by their own parts and remain Self-led, the client's parts feel acceptance despite their extremes and can relax, allowing the client's Self to return and help.

Remaining Self-led, however, is not easy. When you encourage clients to work with their exiles, you invite them to be extremely vulnerable. These exiles become hopeful that they will finally be healed, and they can become very attached to you as their redeemer. You may be idealized in ways that feel uncomfortable to some parts of you but great to other parts. Protectors will watch your every move, waiting for a sign that you are not safe and are looking for an excuse to shut down the process or attack you.

PARTS OF THE THERAPIST

For all these reasons, IFS therapists will be the targets of the extreme parts of their clients. Regardless of why these parts appear, sudden and extreme shifts in clients can be startling and can trigger therapists' protectors. During this stage of client vulnerability, the clients' firefighters are most protective and it is during this stage that therapists can do the most harm if they lose Self-leadership. Vicious cycles can easily escalate between the protectors of the therapist and client, which result in at least temporary breaks in connection. Sometimes this disconnect is obvious to the mindful therapist, who notices negative thoughts about the client or a judgmental tone in his voice. When this occurs and the therapist is able to ask his protectors to relax and step back, his return to Self-leadership may be enough to also help the client's protectors relax. On the other hand, I often try to take advantage of these moments to model speaking for parts. I might say, "I just noticed one of my judgmental protectors reacting to what you just said and I was able to help it relax. I apologize if it triggered you. I wonder if you noticed it and, if so, how you reacted?"

This type of meta-communication validating the dance of parts between intimates can be healing in any relationship and especially so for clients who have had little validation in their lives. Many clients have never experienced conflict with someone after which the other person owned their role and apologized. Instead, they are used to being shamed and then rejected for being so sensitive, emotional or impulsive. Many of my clients have histories of severe abuse, betrayal and neglect, so their protectors do sometimes overreact and view their partners (or therapists) as perpetrators. As a result, they often carry the sense that they are too difficult and are doomed to be alone. It is one of life's great inequities that so many people who get hurt as children are reinjured throughout their lives because the original hurt left them so raw and reactive. They deserve to be close to someone who, after initially being triggered, can regain *perspective* and see behind the client's explosive rage, icy withdrawal or manipulative controlling, to the pain that drives those behaviors. The ability to regain compassionate *presence* in the face of a client's extreme protectors or exiles is the *sine qua non* of a good IFS therapist.

After the fifth session in which Larry had acknowledged the suicidal part, Ann seriously considered not returning. She had left that session with an unfamiliar sense of hope—feeling that maybe some things in her life really could change and maybe she was not as inept as she had always thought. But within a day or two a coalition of self-hating critics tore apart her momentary sense of pride and ridiculed her for daring to think she was good at anything. The suicidal part, feeling duped into believing that it might get some attention before being abandoned by her, was stronger than ever. She felt bereft, worse than if she had never tried therapy.

When she arrived at the sixth session, she was distant and shut down, not looking at Larry and answering all his questions with "I don't know." At some point, Larry decided to stop trying so hard and quit talking, too. They sat together in an icy silence until Ann finally said that she was going to leave.

As she got up, Larry asked her to sit down again. He told her that during the silence he had found a part of his own who was feeling discouraged by how distant she seemed and was taking it personally. He

had been moved and excited by the last session and his part felt disappointed that the change in her had not lasted but, instead, she seemed hostile. He had worked with this part and gotten it to step back. Once it did, he could immediately sense that she was hurting and disappointed.

Larry apologized to Ann for letting that part take over and Ann appreciated the apology. It was true, she said, that when she arrived she was scared by how bad she felt and was afraid to let him in again. His reaction had only compounded her shame and hopelessness because she sensed he was abandoning her. It meant a lot that he cared enough to work on himself and come back to her. She confessed that she was more suicidal than ever and that her critics had started working overtime soon after their last session.

Larry said, “If I wasn’t triggered, I would have realized that the reaction in your system to the hope we both felt last time was very predictable. Your protectors probably knew that you couldn’t afford to get hopeful and then feel disappointed again, so they sabotaged the work and attacked your confidence.”

They agreed to work with her critical protectors in the next session, but until then Larry asked her to try to realize what her protectors were really about and not to take their rants so seriously. She said she thought she could do that. He also told her suicide part to not despair—that they were coming back to it as soon as possible.

BURDENED THERAPISTS

Remaining Self-led in the presence of extreme protectors is not easy because the client is not the only one in the room who has them. Most therapists I know are like me, wounded healers who are heavily burdened from their families and peers. We enter the therapeutic arena full of polarizations among our parts. For example, many of us have our own inner critics who attack us for making mistakes and hate the weakness and neediness of our exiles. These critics are polarized with angry parts who defend our exiles from inside and attack anyone else who judges us. When a client is weak or needy, tells us that we are mistaken or challenges our competence, she inadvertently stumbles into the middle of our internal wars. Some of us were led to be therapists by extreme caretaker parts who developed their self-sacrificial, overly responsible roles as children. These caretaker parts are often polarized with parts who are tired of worrying about everyone else and want people to quit sniveling and leave us alone. This was the case with Larry’s inner system.

Sometimes it does not take much for clients to trigger a part who makes us try too hard or one who gives our clients the message that they want too much from us, that we are frustrated with their demands. Often clients will get these contradictory messages back-to-back, which can be very confusing. There are a myriad of other vulnerabilities and common polarizations inside of therapists that clients can innocently activate. Some clients look or act like dreaded members of the therapist’s family-of-origin, or complain about the things that his or her spouse complains about. Many of my clients have suffered by unwittingly stepping on my internal landmines or wandering into my inner crossfire.

In the book *Love’s Executioner and Other Tales of Psychotherapy*, Yalom (1989) was brutally transparent about his reaction to his first encounter with a client named Betty.

The day Betty entered my office, the instant I saw her steering her ponderous two-hundred fifty pound, five-foot-two-inch frame toward my trim high-tech office chair, I knew that a great trial of countertransference was in store for me.

I have always been repelled by fat women. I find them disgusting: their absurd sidewise waddle, their absence of body contour—breasts, hips, buttocks, shoulders, jaw lines, cheekbones, everything, everything I like to see in a woman, obscured in an avalanche of flesh. ... How dare they impose that body on the rest of us? (pp. 107–108)

Yalom was courageous in acknowledging the prejudice clients can stir in us simply by the way they look or because of their history or diagnosis. But as he noticed one judgmental thought about Betty after another, his solution was to try to eliminate them from his mind.

And so it went: the entire hour with her was an exercise of my sweeping from my mind one derogatory thought after another in order to offer her my full attention. I fantasized Mickey Mouse, the sorcerers apprentice in Fantasia, sweeping away my distracting thoughts until I had to sweep away that image, too, in order to attend to Betty. (p. 109)

Although it may be instinctively tempting to sweep away a disturbing thought that is interfering in the relationship with the client, it is now widely accepted that countertransference feelings require attention and that attending to them is important for the therapy process as well as for the therapist. In my view, IFS offers a practical and highly effective way of working in the moment with countertransference.

I have had to work with many parts who were triggered by clients. Because I have specialized in the treatment of survivors of severe sexual abuse for years, many of my clients have, at times, been highly suicidal, cut their arms or torsos, binged on alcohol or stolen things. At times they also formed a childlike dependence on me, wanting, and sometimes demanding, lots of decision making, reassurance, contact between sessions, information about my feelings for them or about my personal life, and having tantrums when I left town. Or, at other times, they let me know that I was a terrible therapist, that they had received no help from me, that I did not care about them and had a defective character or that I had hurt them in an extreme way. In addition, they sometimes tried to control how I acted and used the threat of suicide or cutting to manipulate me. Some of them suddenly shifted to thought processes that seemed paranoid or schizophrenic, regressed or deluded. For others, a seemingly small provocation would result in almost murderous rage. Some alternately idealized and devalued me with an unpredictability that made my head spin, and they constantly tried to stretch my boundaries by demanding special treatment or violating my privacy. With some, any progress in therapy was always followed by various kinds of self-sabotage or backlash that made treatment with them seem like a Sisyphean nightmare. All of those behaviors evoked parts of me who could be as extreme as Yalom's. Before I was sensitized to the importance of noticing my reactive protectors, I tortured many clients. My Self would disappear and they were left in a face-off with one of my protectors, which would make their own protectors more extreme.

Of course I am not alone in this. Many therapists are triggered by extreme parts in their clients and interpret the emergence of these protectors as signs of pathology. Those therapists become clinical, detached, defensive and controlling. Others react by trying harder to care take, becoming obsessed with

the client and expanding their boundaries beyond their comfort level but finally feeling resentful. I have witnessed therapists resenting clients who test competence and boundaries and who accuse challenging clients of being manipulative. Being around all of that expressed pain can traumatize therapists vicariously and cause them to dissociate. Finally, clients in IFS therapy sometimes find themselves on a roller coaster of highs and lows and many therapists climb on board, getting excited by apparent improvements and equally discouraged by the inevitable backlash reactions they view as setbacks.

In these reactions the therapist winds up closing her heart to her client. The client will sense abandonment and become increasingly protective, either withdrawing or escalating the provocative activity. I have consulted on any number of cases where the therapists' reactions started a vicious cycle that resulted in the client's rapid disintegration, impaired functioning, serial hospitalizations or overmedication. The therapists involved were generally convinced that their actions were justified by the pathology of the client and were often oblivious to their role in creating it. For some clients, just being with a compassionate, confident therapist who could put an IFS perspective on what happened could reverse the cycle and they improved quickly. Others were left so distrusting of therapists that it took a long time to earn their trust. So, as others have written using different language (Herman, 1992; Linehan, 1993), we play a dangerous game when we subscribe to views of clients that emphasize their pathological or manipulative nature, and ignore, minimize or deny the influence of our own parts.

It is not possible to do therapy without having parts take over at times. The level of intimacy required for clients to feel safe in going to their exiles will result in the emergence of all kinds of parts in both the client and therapist. Ideally, when that happens I notice that I have lost Self- leadership and quickly get my parts to step back during the session, as Larry did. Often I will apologize to the client for the intrusion. In this way it's possible to quickly repair breaks in Self-to-Self connections with clients. After the session I try to use the experience to find out why the part felt the need to take over and what it needs to trust my Self in the future. This often leads me to care for exiled parts whom I might not have found otherwise. So therapists can use their countertransference reactions as trailheads for their own healing. This, in turn, makes for one less landmine or crossfire for clients to deal with.

THERAPIST: MONITOR THYSELF

To catch oneself in a part, one needs to have an exquisitely sensitive parts' detector focused on one's own inner family. I am frequently asking myself how I feel toward certain clients and checking the openness of my heart or the energy in my body as I visualize those clients. Different IFS therapists have different ways of noticing the degree to which their Self is present and the reader is encouraged to develop rituals for doing so as well.

Unfortunately we are not always the best detectors of our own parts. We all have blind spots—protectors with whom we are so closely identified or who are so clever that they seem like our Selves. These pseudo-Selves (aka Self-like parts) are extremely difficult for us to detect, even though they may be quite apparent to others, including our clients. Clients often detect these parts and, if they have the courage to mention it, we should heed their warning and take a careful look. Of course it's hard to totally

trust a client's parts detector because of their transference, but I have found more often than not that clients correctly note the absence of my Self even if sometimes they mis-interpret the reasons and overreact.

So, we therapists are not Buddhas and whether we notice it or not we are regularly triggered by the intense interplay with our clients. Therefore, we must keep honing our parts detectors regarding ourselves. Fortunately, as we become increasingly familiar with the physical experience of embodying the Self (McConnell, this volume), we are also better able to notice the somatic and mental shifts when a troubled part hijacks us (we have a "part attack"). With this awareness—and lots of experience doing this type of clinical work—comes the ability to calm the part in the moment and ask it to separate so that Self can return.

GETTING PARTS TO STEP BACK IN THE MOMENT

To illustrate how the therapist works with her own parts, I will return for a moment to the session in which Larry sat in silence with Ann because he was triggered. He scanned his body and found a tension in his face, noting that his heart felt closed. As he focused on those sensations, he found an angry part and asked it to step back. Immediately he felt his heart open. He had compassion rather than resentment for Ann and was free to apologize.

If I had a microphone in my head when I was treating certain challenging clients, you would hear me saying to myself repeatedly something like: "I know you're upset, but just let me stay and handle this. Remember it always goes better if you let me keep my mind open. Just relax and trust me, and I'll talk to you after the session." On good days, those words produce an immediate shift in my level of Self-embodiment—my heart opens, my shoulder muscles release or the crowd of negative thoughts in my head disperses. My client suddenly looks different—less menacing or hopeless and more vulnerable. Between sessions, I will follow up by bringing the parts who my client aroused to my own therapy to give them attention. In this way, our clients become our *tor-mentors*—by tormenting us, they mentor us, making us aware of who in us needs our loving attention.

During the ensuing week, Larry asked his therapist to help him with the angry part who had reacted to Ann's shut down. As he focused on that part, it showed scenes of himself as a 10-year-old futilely trying to cheer up his depressed mother, and he was able to retrieve and unburden that boy. In the next session, he told Ann that he had healed that part so he hoped she would not have to worry about it interfering any more. But he added that if she did sense it, she should tell him.

BEING A SENSITIVE DETECTOR OF CLIENT PARTS

Although therapists are often unaware of their parts, clients, who have not studied IFS, are often even less aware of their parts. As therapists detect client parts, it is tempting to point them out and educate the client regarding their appearance or impact. There are often telltale signs like shifts in tone of voice,

content of conversation, body movement or facial expression. And many therapists come to IFS from traditions in which they were trained to help clients notice those shifts or interpret what they mean.

There are times in IFS therapy where this kind of confronting is necessary and helpful in bringing Self-energy to key parts. There are other times, however, when it will activate the client's protectors in a way that sets the therapy back unnecessarily. You may expose parts whom the system was not yet ready to handle or is not even aware of. Many clients' systems have a wisdom about what to reveal to them and when. Therapists who point out evidence of parts in a well-meaning attempt to move the therapy along, or to bypass resistance, can violate this wise pace. My experience, more often than not, is that if I remain Self-led and keep things safe, the client will find those parts on her own when the time is right. Ironically when resistant protectors are treated with respect rather than as the enemy, the doors often open with surprising speed.

On the other hand, IFS therapists do often operate as parts detectors for clients by helping them remain Self-led while they work inside. For example, when a client is trying to get to know a part, I am constantly pointing out if she is blended with a polarized part, and I ask her to get the second part to step back. I do the same with couples—I catch the parts of each as they pop up. My point is that it is one thing to help a client become aware that she is blended with a protector when you have an explicit contract to keep her in Self, but quite another to point out a movement or body posture that may well be a manifestation of an exile whom the system wants to keep hidden. By having the client focus prematurely on a feared trailhead, you show her protectors that you are not safe and should be resisted in the future. Or, worse, protectors punish the client because you encouraged her to violate their rules.

Nevertheless, there are situations where, to expedite things, I will point to evidence of parts. As I said earlier, I rarely hesitate to identify signs of a client's protector and I also will help clients notice potential exiles when I already have a strong relationship with the client's protectors, or when I believe the client's system could handle the consequences even if some protectors do not agree. When I do spotlight a trailhead that could lead to an exile, I do so in the form of a question, for example, "I noticed that you just flinched. Could you focus on that sensation and see what that's about?" In this way, I have no need to make an interpretation and the client remains the expert on her experience. My choice of whether to focus on such evidence is intuitive and based on knowing the client. The main point is that in IFS we try to be ecologically sensitive and we do not feel compelled to point out signs of parts just because we notice them.

CHALLENGING

Having said all this, therapists are often struck when watching tapes of me working by how strongly I challenge some clients and their parts. These therapists were assuming that being Self-led meant never having to challenge but instead always remaining receptive and empathic. They forget that courage, clarity and confidence are important qualities of Self-energy. When I sense it would be helpful, I do not hesitate to be forceful. The key is to be forceful from Self rather than from your assertive, judgmental manager or pseudo-Self.

I might tell a protector that I do not believe what it is saying and I want it to be direct and honest. Or I might insist that the client get certain protectors to go to a waiting room and remember that they are just scared children. My goal in these instances is to demonstrate to the client that regardless of how imposing, adult-like or convincing their protectors seem, these parts respond to firmness and strong leadership just as out-of-control children do. You can tell whether you are Self-led in these instances by checking on how attached you are to winning any power struggle that might ensue. When in Self, you can be forceful without being attached to the outcome, remembering that it is the protector's right to resist and that they often know better than you about the negative consequences that could arise if the client goes further.

Although most of us do not have a lot of practice, there is an art to being forceful while remaining Self-led. We grow up relying on various protectors to handle the moments of conflict or assertiveness. When you can be assertive from Self, clients not only experience your confidence that they can handle whatever comes, they also sense that you care and that you remain connected even as you challenge them. For many clients such experiences represent the first time anyone has believed in their ability to handle something (i.e., dreaded memories, strong affect, standing up for themselves with someone), and the first time they have been pushed without feeling criticized or disconnected.

Larry's invitation to Ann to speak about his part when she sensed it, made several of her protectors relax. They felt more in control of the therapy and less as if he were the big expert and she were the crazy patient. Instead he was saying they both had parts who could interfere—they were both human. The fact that Larry was committed enough to spend time on what had happened between them in his own therapy reinforced Ann's burgeoning belief that he actually did care. The problem, however, was that if she really believed he cared, he would have a level of power to hurt her that her protectors had vowed never to give another person. So they remained on high alert while agreeing to let her return to her suicidal part. Her protectors all wanted it to work even though they were terrified that it would result in another disappointment.

At first the suicidal part (the keening woman) ignored Ann's attempts to reengage. Larry helped Ann remain persistently caring until she fell back into Ann's arms. Ann asked her what had happened in the past to make her want to leave this world so badly and the woman showed Ann scenes of her mother's funeral when she was 12-years-old. With her mother gone, the girl was totally dependent on her violent father—there was no more escape from him. Ann was able to enter the scene, stand up to the father for the girl, take the girl out of that time to a safe place and help her unburden the terror and despair. Larry was moved to tears while witnessing this work. He felt so much for the 12-year-old and also was proud of how courageously Ann helped her. He did not hide his reaction and, on seeing it, Ann once again could not deny that he cared for her. Their connection was solidifying. When Ann's protectors saw that the girl was now OK and they no longer had to prevent disappointments that could lead to suicide, they became interested in unburdening and taking on new roles.

In a later session, Ann talked about the man she visited periodically for sex. She said she enjoyed being with him even though he only wanted to be serviced by her and would not do anything to take care of her sexually. Larry replied, "Are you sure all of you enjoys it?" Despite Ann's assurances that she was happy spending time with this guy, he kept challenging her. Finally he said, "Why don't we just check inside and see if what you're saying is true?"

Ann agreed reluctantly, and Larry had her start by finding the part who had been talking up to that point in the session. Ann saw a tough teenager and asked her why she liked servicing this man sexually. She replied that she liked being in control of him—he was totally under her power for a period of time and she could do whatever she wanted. After considerable negotiating, the teenager agreed to let Ann get to know the vulnerable parts whom she had to lock up to be with the man. Ann found several younger ones who let her know how humiliated they felt every time she was with him. Ann returned to the tough teenager and told her that it was not OK to incarcerate the young ones so she could have fun. The teenager dropped her guard and showed Ann more scenes of abuse by the neighbor's son and how much she had wanted to be in control with him. After retrieving that girl, the teenager was softer and Ann stopped seeing the man.

Then Ann's self-hating part began to attack her for having serviced this man for so many years, saying, "You are worse than a whore because you didn't even get paid." Larry had Ann ask this part the standard question, "What are you afraid would happen if you didn't call her those names?" It replied that it did not fear anything—it was just speaking the sorry truth. Larry decided he would challenge the self-hating part and asked to talk to it directly. The following conversation ensued:

"Where did you get the idea that Ann is a whore?"

"It's not an idea—it's who she is and she proved that for years."

"I get that doing that with him really bothered you, but Ann is not a whore, so let me ask you again. Where did you get that belief?"

"From her mother."

"OK, I appreciate your being honest about that. Can you let us know how the mother gave you that idea about Ann?"

The self-hating part then showed Ann a scene from the period of time when she was being abused by the neighbor in which her mother found her masturbating and called her a whore. After Ann was able to retrieve and unburden that shamed girl, the self-hating part gave up the belief that Ann was a whore.

I use this form of challenging frequently when a protector attacks a client repeatedly for reasons that do not seem to be protective but instead seem to reflect a core belief. Just as postmodernists deconstruct truths that seem god given, asking parts where they got beliefs that just seem to be true can help them realize that those beliefs actually came from a person or event and this helps the part let the belief go.

THE TRANSFORMATIVE POWER OF SELF

While I have discussed many different aspects of the relationship between the IFS therapist and client in this chapter, the main theme can be summarized simply. Self-energy has tremendous transformative power. When the client experiences the compassion and acceptance of the therapist's Self, she heals. When the clients' parts experience the compassion and love of the client's Self, they heal. When the client and therapist connect Self-to-Self, they both heal. There is a transpersonal dimension to all of this

that I sense strongly but do not often articulate. Others have also recognized this aspect of healing and describe it similarly and eloquently. Consider, for example, these words from Pendergast, Fenner, and Krystal (2003) regarding what happens when therapists can access this state:

The result is the emergence of a natural simplicity, transparency, clarity, and warm acceptance of whatever arises within themselves and their clients (p. 3) ... Attention drops from the head to the heart. Without any conscious intention on the part of the therapist, an optimal field of loving acceptance arises that facilitates transformation. What has been unmet is waiting to be fully embraced before it can transform. ... We are no longer problem solvers facing problem holders. Instead, we are Being meeting itself in one of its infinite and intriguing disguises. (pp. 7–8)

Toward the end of his career, Rogers (as cited in Pendergast et al., 2003) also put it beautifully and succinctly:

I find that when I am closest to my inner, intuitive self, when I am somehow in touch with the unknown in me ... whatever I do seems to be full of healing. Then, simply my *presence* is releasing and helpful to the other. There is nothing I can do to force this experience, but when I can relax and be close to the transcendental core of me ... it seems that my inner spirit has reached out and touched the inner spirit of the other. Our relationship transcends itself and becomes a part of something larger. Profound growth and healing and energy are present. (p. 93)

So the IFS bumper sticker could be: “Self Heals.” The essence of the therapeutic relationship is in this ineffable touching of spirits, Being meeting itself, the Self-to-Self connection. Humanists, transpersonalists, Buddhists and some current analysts use different words to describe it, but we are all getting at the same essence. IFS is but one map to that sacred place. It is, however, a clear, nonpathologizing, empowering user-friendly map. It is relational in multiple dimensions and promotes healing connections at all levels of the client’s system, most crucially between the client’s Self and parts but also between the therapist’s Self and the client’s whole system.

Thus I have come full circle on the importance of the therapeutic relationship, from minimizing its role to making it a foundational element of IFS therapy. As I mentioned earlier, psychotherapy research has shown that the therapeutic relationship is key. Through IFS, therapists are given clear-cut ways to ensure that the relationship is filled with Self-energy. The loving, respectful, supportive, challenging and accepting manner in which the therapist relates to the client accesses her Self, who then relates that way to her parts. When the therapist notices, listens to and speaks for her own parts from Self, the client opens to this new way of being in the inner and outer world. The sacred connection they form transcends the therapy hour. It facilitates unburdening and bolsters a lifelong practice of learning to trust Self-leadership for both client and therapist.

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